

Your Clinicians Spend Two Hours on Paperwork for Every One Hour With a Patient.

A data-driven look at where clinical time actually goes, what it costs your practice every year, and how ambient AI is already recovering 5+ hours a day for clinics like yours — starting this quarter.

40%

of clinician working hours consumed
by administrative upkeep

49%

industry-wide clinician burnout linked
to documentation load

\$31B

spent annually on prior-authorization
paperwork alone

The EHR Paradox: A Tool Built for Efficiency Is Now Your Biggest Time Sink

Electronic health records were introduced to save time. Instead, across primary care, physical therapy, and allied health clinics, they have become the largest single drain on clinical capacity — and the largest source of physician and therapist burnout in the industry.

2:1

Hours spent on EHR data entry and paperwork for every hour of direct patient care

15–20

Hours per week outpatient physicians lose to non-clinical admin tasks

56%

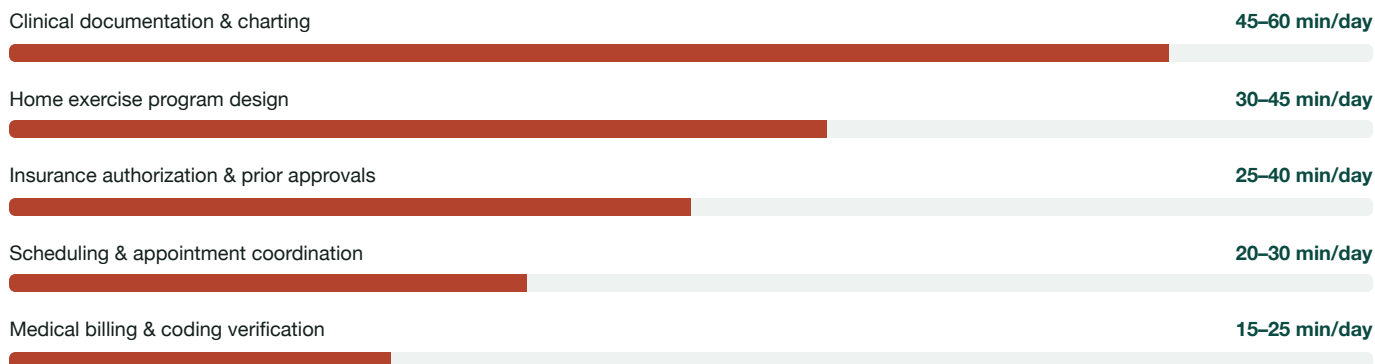
Of patients say their clinician seems distracted by paperwork during visits

Clinicians now report spending up to **40% of their total working hours** on administrative upkeep — much of it completed after clinic doors close. This isn't isolated to large hospital systems: Canadian family physicians alone log an estimated **18.5 million hours per year** on paperwork, the equivalent of **55.6 million patient visits** that are never seen because the time was spent typing instead of treating.

"22% of patients note that their provider focuses primarily on a computer screen during the visit" — a direct, measurable erosion of the clinician-patient relationship your practice depends on.

WHERE A CLINICAL DAY ACTUALLY GOES (ALLIED HEALTH & WELLNESS CLINICS)

In a typical 8-hour shift, physical therapy and wellness clinicians lose 49% of the day to non-clinical tasks — leaving only 4–5 hours for the patients they were hired to treat.



67% of physical therapists cite this paperwork load as their primary source of job-related stress — a leading driver of staff turnover across the industry.

Ambient AI Scribes Cut a Patient Encounter From 41 Minutes to 16

Across a multisite clinical study spanning five major U.S. academic medical centers, clinicians using ambient AI scribes reduced total EHR time by 13.4 minutes and charting time by 16.0 minutes per patient encounter — without changing a single thing about how they treat patients.

TRADITIONAL WORKFLOW

Patient visit — 15 min

Manual charting & coding — 16 min

Administrative upkeep — 10 min

Total cycle: 41 minutes / patient

AI-OPTIMIZED WORKFLOW

Patient visit with ambient AI listening — 15 min

Click to approve AI-drafted note — 1 min

Automated billing verification — background

Total cycle: 16 minutes / patient

WHAT THIS MEANS FOR A CLINICIAN SEEING 20 PATIENTS A DAY

Time Recovered = Daily Encounters × Minutes Saved per Encounter (16 min, per JMIR Medical Informatics multisite study)

5.3 hrs

recovered clinical capacity every single day — reallocated to patient care, research, or simply going home on time

+0.49

additional patient visits per week once documentation time is reclaimed

+10.6%

increase in direct eye-contact time with patients after adoption

BEYOND CHARTING: AUTOMATING THE PRIOR AUTHORIZATION BOTTLENECK

Prior authorization alone costs the U.S. healthcare system **over \$31 billion annually**, with physicians spending more than 13 hours a week navigating manual approvals. AI-enabled systems now automate clinical data extraction directly from the EHR and auto-populate payer-specific compliance forms — automating up to **75% of manual administrative tasks** and cutting approval turnaround from 5–7 days down to 24–48 hours.

Clinical Process	Traditional Manual Time	AI-Automated Time	Real-World Impact
Clinical note preparation	16.0 min / encounter	1.0–2.0 min / review	13.4–16.0 min saved per encounter
Prior authorization submission	3–5 days processing	Real-time to 24 hrs	80% staff labor reduction
Patient care ratio (8-hr shift)	4.0 hrs direct care	6.5–7.0 hrs direct care	2+ hours reclaimed daily per provider
Home exercise program creation	15.0 min / program	Under 5.0 min	Automated template matching
Clinical scheduling coordination	20–30 min daily	Autonomous background action	30–45% reduction in no-show rates
Claims denials & RCM audits	15–25 min daily	Under 2.0 min verification	30% denial reduction; 98%+ coding accuracy

The Real Question Isn't Whether You Can Afford This. It's Whether You Can Afford to Wait.

Every week your clinicians spend on manual documentation instead of patients is billable capacity, staff morale, and patient trust you can't get back. The clinics adopting ambient AI and automated prior-authorization workflows today are compounding an advantage that gets harder to close every quarter.

1 Ambient AI scribes

Securely record the clinician-patient conversation and generate a structured, compliant clinical note directly in your EHR — reducing charting time by 16 minutes per encounter.

2 Automated prior authorization

Extracts data straight from the chart and auto-populates payer forms, cutting turnaround from days to hours and lifting first-pass claim acceptance from ~70% to 92%.

3 Predictive scheduling & RCM automation

Reduces no-show rates by 30–45% and flags coding errors before submission — protecting margins without adding headcount.

SOLUTIONS REFERENCED IN THIS SECTOR

Ambient AI Scribes

Automated Prior-Auth Engines

Predictive Scheduling

RCM & Coding Automation

EHR-Integrated NLP

codemypixel.com Builds This — Custom-Fitted to Your Clinic

We design and build automation systems that plug directly into your existing EHR and billing workflow — ambient documentation, prior-authorization automation, and intelligent scheduling — so your clinicians get their time back without changing how they practice medicine. **No off-the-shelf guesswork. Built around your patients, your payers, your workflow.**

[Talk to codemypixel.com about your clinic →](#)

Sources synthesized from: JMIR Medical Informatics (Impact of an Ambient AI Scribe Among Clinicians and Patients, 2026) · Mass General Brigham Newsroom · Innovaccer · Physitrack Insights · Healthcare Dive · Tandem Health · Aspect Billing Solutions · PMC (Burden of Administrative Responsibilities in Primary Care) · The Medical Futurist · Carevyn.ai. Figures represent industry research aggregated for illustrative planning purposes; actual results vary by practice size, payer mix, and system integration.